

≥ 2 SIRS + suspected/present infection

3 causes

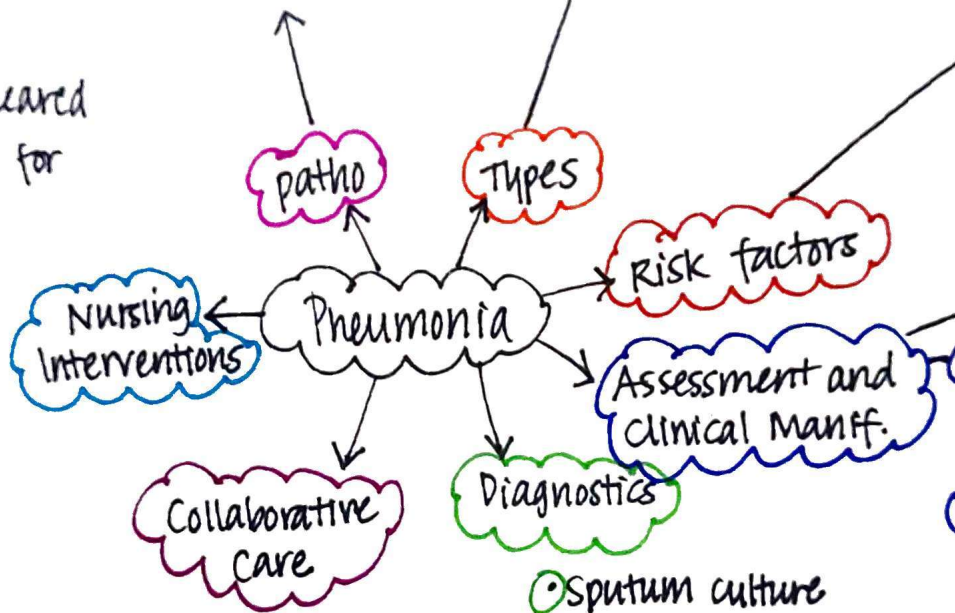
SEPSIS
T > 100.4 or < 96.8
RR > 20
HR > 90
WBC > 12,000 or < 4,000 or > 10% bands

- Aspiration, Inhalation, hematogenous spread
- Acute lung infection of bronchioles, alveoli/ducts
- Inflammation occurs (pulmonary edema)

- Community Acquired
- Hospital Acquired
- Aspiration
- Opportunistic
- Viral/Fungal
- Necrotizing

- Immunosuppressed
- Decreased LOC
- Intubation
- Older age
- Immobility
- IV drug abuse/smoking

- Airway patent and cleared
- Do not delay antibiotics for culture results
- Implement preventative practices (aspiration precautions, coughing/deep breathing, ambulation, IS)
- Hydration/Nutrition promoted
- Rest/energy conservation, positioning
- Administer meds
- Educate on knowledge of disease/monitor for complications
- Health promotion (smoking cessation, vaccines, hand hygiene, avoid sick people)



• Confusion
• U-BUN > 20
• Respiration ≥ 30
• BP - SBP < 90 or DBP ≤ 60
• > 65 age

- O2 therapy
- Antibiotics, analgesics, antipyretics, steroids
- Adequate hydration & nutrition
- Rest and activity as tolerated
- Respiratory Therapy

- Sputum culture and sensitivity
- CBC/ABGs
- Chest Xray
- C-reactive protein

- Breath sounds
- rhonchi, crackles, ↓ breath sounds, ↑ fremitus
- Cough, dyspnea/tachypnea, fever/chills, pleuritic chest pain, sputum, fatigue/confusion, v/s
- Assess for risk factors/cause for pneumonia
- Older Adults: initial/progressive symptoms may include: ↓ LOC, confusion and hypothermia

Complications: sepsis, pleuritis, pleural effusion, atelectasis, pneumothorax, acute respiratory failure